SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Weekly cetuximab + head and neck radiotherapy

Locally advanced squamous head and neck cancer in people ineligible for platinum-based treatment. · NCCP 00207 · version 6 · 7-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives.

Schedule
Cetuximab starts one week before radiotherapy: a 400 mg/m² first infusion, then 250 mg/m² every week during radiation until radiotherapy ends. The number of weekly doses follows the actual radiotherapy plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cetuximab
EGFR-TARGETED ANTIBODY

- ☐ Infusion reactions including anaphylaxis
- ☐ Acne-like rash, dry skin and nail infection
- ☐ Low magnesium and other mineral changes
- ☐ Eye inflammation
- ☐ Diarrhoea and dehydration
- ☐ Rare serious lung inflammation

Radiotherapy
LOCAL TREATMENT

- ☐ Sore mouth and throat
- ☐ Dry mouth, taste and saliva changes
- ☐ Swallowing or voice problems
- ☐ Skin reaction in the treated area
- ☐ Jaw stiffness and dental problems
- ☐ Some swallowing, saliva or tissue effects can be lasting

Discuss the individual aim, alternatives and uncertainty; serious or lasting harms; treatment burden; fertility and pregnancy precautions; and the patient's priorities. Explain infusion anaphylaxis, rash and nail/eye problems, mineral loss and post-treatment testing; the observation plan requires reconciliation of NCCP and product information. Discuss radiation mouth, skin, swallowing, saliva, dental and jaw effects, including lasting effects and potential nutritional support. Radiation has its own consent process. Confirm actual urgent contacts and teach-back; this draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00207 v6.

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Developed by Dr Paul O'Brien